

Patient Name: 
Patient Number: 

Date: 
Date: 

INITIAL ONCOLOGY CONSULT

Primary Diagnosis:
Secondary malignant neoplasm of bone
Malignant neoplasm of unspecified part of right bronchus or lung

Other Medical Problems:

Date	Type	ICD-9	ICD-10	Problem	Comment	Status
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Reason for Today's Visit:
8/18/2025:
Stage IV right lung cancer with metastasis to lymph nodes and spine. Patient is accompanied by his wife.

History of Present Illness
 and two locations in the spine.
Patient reports occasional cough but denies headache, dizziness, or lightheadedness. He reports progressive weight loss of approximately 25 lbs over the past three years and increased fatigue. Recent CT of brain and PET scan were performed showing disease in the right lung, cervical lymph nodes, and spine.

Past Surgical History
- Cardiac bypass (date unspecified)
- Treatment for rectal cancer in 2001 (hospitalized for one week with chemo and radiation, then discharged for one month, then readmitted for another week of treatment at SIUH)

Social History
- Current smoker
- Recently stopped alcohol consumption (previously consumed 1 drink, 3 times per week)
- Family: Has two daughters
Patient is married.
Patient lives with spouse.
Current Smoker 1PPD since age 11
Advised to Quit Patient reports former alcohol consumption.
Denies any illicit drug use.
contractor-retired
Patient has had the following occupational exposure: . Asbestos

Family History
- Mother: Lung cancer (diagnosed shortly before death in 1991-1992), was a smoker
- Multiple family members: Hereditary hearing issues



Mother lung cancer, smoker

Personal History

Patient denies having ever received blood products in the past.

Allergies

- No known medication allergies

Allergies

No Known Drug Allergies

Medications

Inside	Drug	Script Date	Qty	Rfls	Instructions
	ALBUTEROL HFA 90 MCG INHALER	11/1/2024	20.1	0	INHALE 1 TO 2 PUFFS BY MOUTH EVERY 4 TO 6 HOURS AS NEEDED
	AMLODIPINE BESYLATE 5 MG TAB	4/1/2025	180	0	TAKE 1 TABLET BY MOUTH TWICE A DAY
	ATORVASTATIN CALCIUM 40 MG TABS	8/11/2025	90	0	
	CARVEDILOL 6.25 MG TABLET	2/24/2025	180	0	TAKE 1 TABLET TWICE A DAY WITH MEAL ORAL TWICE A DAY
	ENALAPRIL MALEATE 10 MG TABS	11/20/2024	90	0	
Y	Keytruda 25 mg/mL intravenous solution	8/18/2025	8	6	INFUSE 200MG INTRAVENOUSLY EVERY 21 DAYS
	NITROGLYCERIN 0.4 MG TABLET SL	4/1/2025	100	0	USE 1 TABLET SUBLINGUALLY AS NEEDED FOR CHEST PAIN

Review Of Systems

Constitutional: +fatigue, +weight loss, -fever

HEENT: -headache, -dizziness, -lightheadedness

Respiratory: +occasional cough, -shortness of breath, -wheezing

Cardiovascular: -chest pain

Gastrointestinal: -acid reflux, -abdominal pain, +lactose intolerance, -nausea, -vomiting, +1-2 bowel movements daily

Genitourinary: +nocturia 1-2 times per night

Musculoskeletal: +back discomfort with bending

Neurological: -dizziness, -lightheadedness

Psychiatric: -anxiety, -depression

Vitals

Vitals on 8/18/2025 1:44:00 PM: Height=67in, Weight=105lb, Temp=97.2f, Pulse=84, SystolicBP=112, DiastolicBP=67, O2 Sat=98%

Physical Exam

General: Normal presentation. In no acute distress.

Skin: No rash noted.

Eyes: Not examined

Ears, Nose, Mouth, & Throat: Not Examined.

Neck: No lymphadenopathy noted.

Cardiovascular: Regular rate and rhythm. S1, S2. No murmurs. No extrasystole.

Breast: Normal

Chest/Respiratory: No dyspnea.

No accessory muscle use.
Digital Rectal Exam: Not examined.
Abdomen: Soft. Non-tender to touch, no abdominal pain. No guarding noted.
Genito-Urinary: Deferred.
Hematologic/Lymphatic: No petechiae noted. No purpura noted. No lymphadenopathy noted.
Musculoskeletal: No kyphosis. Right arm length shorter then left with smaller fingers, looks like congenital defect
Neurological: Affect normal. No tremors observed.
Psychiatry: Oriented to person/place/time. No anxiety observed during exam. No signs of depression noted during exam. Intact short-term memory. Intact long-term memory.
Extremities: No clubbing noted to fingers or toes. No cyanosis noted. Capillary refill less than two seconds. No edema.

Patient reported pain
Pain Scale 2 - Discomforting

Treatment Recommendations:
Reassess pain at next visit.

ECOG
Performance Status -ECOG 1: Restricted in physically strenuous activity but ambulatory and able to carry out work of a light or sedentary nature, e.g., light house work, office work

Labs
No lab results have been entered for the visit date.

Labs
Lab results on 8/18/2025: WBC=7.2 x10³/UL, RBC=4.50 x10⁶/UL, Hgb=13.2 g/dl, HCT=41.2 %, MCV=91.6 fL, MCH=29.3 pg, MCHC=32.0 g/dL, RDW [Ratio]=13.5 %, MPV=10.5 fL, Plat=161 x10³/UL, ANC=4.6 x10³/UL, Lymph#=1.4 x10³/UL, MONO#=0.7 x10³/UL, BASO#=0.1 x10³/UL, EOS#=0.4 x10³/UL, Gran%=64.5 %, Lymph%=19.0 %, MONO%=9.8 %, BASO%=0.7 %, EOS%=5.9 %

Radiology Reports

Print? ☐	Date of Doc.	Name	MD Interpretation	Comment
☐	7/7/2025	CT CHEST WITHOUT IV CONTRAST		
☐	7/15/2025	PET CT FDG SKULL BASE MID THIGH WITHOUT IV CONTRAST		
☐	8/15/2025	CT BRAIN WITH IV CONTRAST		

Pathology Reports

Assessment:
[REDACTED]

Patient was previously informed he had stage IIIA lung cancer, but imaging confirms stage IV disease with metastasis to lymph nodes in the neck and two sites in the spine. Recent imaging includes PET scan and CT of the brain. The brain CT was negative for metastatic disease. Genetic testing of the tumor has been performed and indicates immunotherapy alone would be the optimal treatment approach without need for chemotherapy or radiation.

The patient has a history of rectal cancer in 2001 treated with chemotherapy and radiation at SIUH.

Patient is experiencing occasional cough, progressive weight loss (25 lbs over 3 years), and increased fatigue. He is a current smoker and recently discontinued alcohol consumption. He has developed nocturia over the past few months and has chronic back

discomfort with bending.

Recommendation/Plan:**1. Stage IV Lung Cancer**

- Immunotherapy treatment plan: One-hour infusions every three weeks
- Starting treatment on Wednesday if possible
- Immunotherapy selection based on favorable genetic testing results showing no need for chemotherapy
- Will monitor for immunotherapy side effects: increased cough, increased shortness of breath, or >3 loose bowel movements
- Patient instructed to take two Imodium tablets and call office immediately if diarrhea occurs
- Consider MRI of brain with Valium premedication for claustrophobia if patient agreeable, patient is not ready to schedule at this time
- Discussed prognosis with patient and family; his daughter was on the phone today for his visit, approximately 80% success rate for disease remission with immunotherapy

2. History of Rectal Cancer

- Confirmed history of rectal cancer treatment in 2001 at SIUH
- No active issues related to previous cancer noted today

3. Health Maintenance

- Tobacco cessation strongly encouraged given lung cancer diagnosis
- Recent alcohol cessation acknowledged and encouraged to continue

PLAN SUMMARY:

- Start immunotherapy infusions every three weeks for stage IV lung cancer
- First infusion scheduled for Wednesday
- Consider brain MRI with Valium premedication if patient willing, he was not willing to schedule today
- Monitor for immunotherapy side effects (increased cough, shortness of breath, diarrhea)
- Second opinion at Sloan Kettering offered if desired
- Tobacco cessation strongly encouraged

Treatment Plan:

Immunotherapy

Diagnosis Plan:

Order Tests: *CBC/Diff/Platelets, *CEA, *B-12, *T-4 Thyroxine (Total), *TSH, *Ferritin, *FE/Iron, *Total Iron Binding (TIBC)/UIBC, *Comp Metabolic Profile (CMP), Guardant360. Side-effects were reviewed and appropriate measures of support will be provided. We had discussed the composition of chemotherapy regimen, length of chemo-cycle, duration of treatment and the time to assess response to treatment.

Documents & Plan Reviewed

Patient's medical records were reviewed Pathology results reviewed and explained in full Radiology imaging and dictated report were reviewed; the disease status and level of response to the treatment were explained in great detail

Care Coordination & Patient Education

We spent sufficient time to discuss many aspects of care; questions were answered to patient's satisfaction. The diagnosis and care plan were discussed with patient in detail.

Schedule follow-up in:

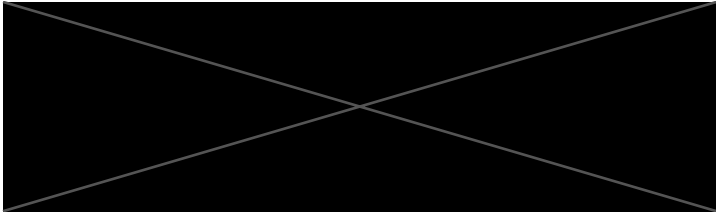
4 weeks.

Attestation

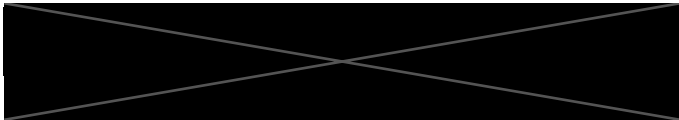
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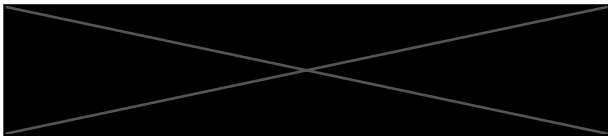
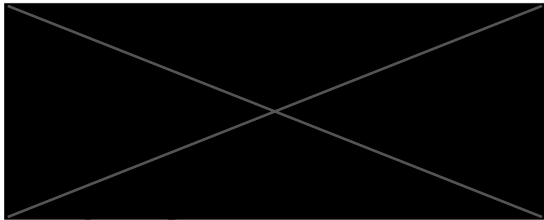
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Signe



Signed





Patient Name: 
Patient Number: 
Report Date: 
Laboratory Use Only: 

DOB: 
Sex: 

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8/18/2025

Comments:

	Result	Value (Previous)	Units	Range	Lab
CBC -Final <i>Ordered by: Seetha Murukutla</i>					
	WBC	7.2	x10 ³ /UL	3.8 - 10.8	SIRICH~AV
	RBC	4.50	x10 ⁶ /UL	4.50 - 6.50	SIRICH~AV
	Hgb	13.2	g/dl	13.0 - 17.0	SIRICH~AV
	HCT	41.2	%	40.0 - 54.0	SIRICH~AV
	MCV	91.6	fL	80.0 - 100.0	SIRICH~AV
	MCH	29.3	pg	27.0 - 33.0	SIRICH~AV
	MCHC	32.0	g/dL	32.0 - 36.0	SIRICH~AV
	RDW [Ratio]	13.5	%	11.0 - 16.0	SIRICH~AV
	MPV	10.5	fL	6.0 - 12.5	SIRICH~AV
	Plat	161	x10 ³ /UL	150 - 500	SIRICH~AV
	Gran%	64.5	%	38.0 - 80.0	SIRICH~AV
	Lymph%	19.0	%	15.0 - 51.1	SIRICH~AV
	MONO%	9.8	%	0.0 - 13.0	SIRICH~AV
	EOS%	5.9	%	0.0 - 8.0	SIRICH~AV
	BASO%	0.7	%	0.0 - 2.0	SIRICH~AV
	ANC	4.6	x10 ³ /UL	1.5 - 7.8	SIRICH~AV
	Lymph#	1.4	x10 ³ /UL	0.9 - 3.9	SIRICH~AV
	MONO#	0.7	x10 ³ /UL	0.1 - 1.0	SIRICH~AV
	EOS#	0.4	x10 ³ /UL	0.0 - 0.5	SIRICH~AV
	BASO#	0.1	x10 ³ /UL	0.0 - 0.2	SIRICH~AV

Performing Location:
SIRICH = MED/ONC - Staten Island - 2801 Richmond Ave, 2801 Richmond Ave, , Staten Island NY

-- End of Report --

